



MODULE 3 — MEDICAL VIRTUAL ASSISTANT FUNDAMENTALS

AfyaDesk Remote Medical Careers Course | The Daily Work That Gets You Hired and Retained

Estimated study time: 8–10 hours (reading + 7 simulations + recordings)

Prerequisites: Modules 1–2 (roles shortlist + reliability system with EOD template)

What you will produce: 7-simulation portfolio (bookings, reschedules, reminders, inquiry call, escalation log, no-show follow-ups, 2-doctor calendar) + MVA SOP cheat-sheet.

Learning Objectives

By the end of this module, you will be able to:

1. Define the MVA role, responsibilities, and hard clinical boundaries with employer-safe language.
2. Map the 9-step medical office workflow and name your ownership and KPIs at each step.
3. Communicate with patients professionally across phone, SMS, email, and portal while protecting privacy.
4. Schedule new, follow-up, urgent, and telehealth visits accurately with visit-type durations, buffers, and dual-zone confirmations.
5. Manage one and two-doctor calendars without conflicts, with daily prep emails and waitlist filling.
6. Execute 24-hour, 3-7-14-day, and 90-day follow-up cadences that cut no-shows and close care gaps.
7. Coordinate referrals end-to-end from order to closed loop with tracker discipline.
8. Triage inboxes, manage email/telephone load, and onboard new patients to 95% completeness.
9. Escalate red-flag clinical concerns in under 15 minutes without giving medical advice.
10. Complete seven practical simulations to hiring standard with logs and reflection.





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1. Role of a Medical Virtual Assistant: Air Traffic Controller, Not Pilot

A Medical Virtual Assistant is the remote front desk, coordinator, and inbox manager for a practice. Doctors fly the plane (diagnose, prescribe, decide). You prevent collisions: double-books, lost referrals, unanswered urgent messages, angry patients leaving 1-star reviews.

Hospitals in Kenya have receptionists, records clerks, and appointment desks in one building. International small practices (1–5 doctors) combine all three into you, working from Nairobi, Kisumu, or Mombasa via EHR, phone system, and Slack.

1.1 What you DO (own these proudly)

- Schedule, confirm, remind, reschedule, fill cancellations from waitlist in priority order.
- Register patients: demographics, contacts, emergency contact, language needs, insurance front/back photos, consent, portal invite.
- Verify admin details: eligibility active? referral needed? in-network specialist? forms complete?
- Triage messages: admin (address change, appointment) handle; clinical (new symptoms, side effects, refill with red flags) route urgently per SOP — never diagnose.
- Chase referrals and prior authorizations: send packet, confirm receipt, book patient, track attendance, retrieve consult note, close loop.
- Prepare charts: tomorrow's list — missing forms, unverified insurance, unsigned orders flagged by 4pm day before.
- Send forms, links, summaries: telehealth link + tech checklist, post-visit summary within 2 hours, lab slips with instructions.
- Document admin actions factually in EHR with timestamp + initials; update trackers and boards.
- Report daily: EOD with numbers (confirms, no-shows filled, referrals sent/closed, inbox zero status, blockers).

1.2 What you DO NOT (hard boundaries that protect license and job)

You do not diagnose ("That rash looks like eczema"), prescribe or change doses ("Take two instead of one"), interpret labs or imaging ("Your sugar is a bit high but okay"), reassure medically ("That chest pain is probably gas"), override doctor decisions, or disclose results before clinician release.

Why boundaries matter: a well-meaning "Don't worry, it's normal" that delays ER for MI can kill and ends your contract plus legal exposure. Employers test boundaries in interviews with "What would you tell a mother whose baby has 39°C fever and asks if she should come now?" Correct: follow fever/urgent SOP + escalate immediately + advise urgent care/ER per protocol,



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document time — not “Give Panadol and wait.” Module 4 and 8 give exact scripts. Memorize:
Understand medicine deeply, work in administration strictly.

1.3 How to talk about scope in interviews

“I support clinical care administratively. I understand terminology to schedule correctly and document accurately, but I never diagnose, prescribe, or give medical advice. For any clinical question I flag urgently to the clinical team per SOP, advise the patient on how to seek urgent care if red flags, and document time and action. Patient safety comes from fast escalation, not guessing.” This single paragraph wins trust.

Pause and reflect: Write 3 DOs from your past Kenyan experience that transfer (e.g., “Queued 80 OPD patients by urgency”) and 3 DO NOTs you will never do remotely. Keep for CV.

2. Medical Office Workflow: The 9-Step Patient Journey You Own

Memorize this flow — every task maps to it, every bottleneck costs money and trust:

1. **Inquiry:** Call, website, portal, referral in. You answer in 3 rings, capture need, verify contact, offer next available that fits urgency + insurance.
2. **Registration + insurance:** Full legal name, DOB, sex, phone + backup, email, address, emergency contact, ID, insurance (member ID, group, payer phone, front/back photos), PCP, allergies, language, consent. Verify eligibility day before — active? deductible? referral needed? in-network?
3. **Scheduling:** Correct visit type + duration + provider + location/link + zone. New 40 min, follow-up 20, urgent 15 buffer, procedure 60, telehealth needs link + tech check.
4. **Reminders:** 48-hour + 24-hour + same-day SMS/email/call per SOP. Include date, time with zone, provider, link/address, prep, callback, reschedule option. Track confirms.
5. **Check-in / pre-visit:** Forms received? ID/insurance uploaded? copay collected? vitals/history updated? Chart prepped? Telehealth link tested?
6. **Visit (clinician owns, you support):** Monitor waiting room, message delays (“Dr. Lee running 10 min late — still with you”), troubleshoot tech, take admin notes.
7. **Post-visit:** Summary sent in 2 hours, e-prescription routing confirmed, lab/referral orders launched, follow-up booked before patient leaves (virtual or physical).
8. **Billing / claims support:** Eligibility re-verified, codes from clinician attached, claim status tracked, denials triaged, patient-friendly balance calls.
9. **Recall / follow-up:** 24-hour check, lab/referral chase Day 3-7-14, chronic 90-day recalls, annual physicals, satisfaction survey.





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Your fingerprints are on 1–5 and 7–9. Visit quality means nothing if Step 4 fails (no-show) or Step 7 fails (referral leaks). US practices lose \$200,000+/year on leaking referrals and 14–18% no-shows. Cutting no-shows 16%→8% adds ~25 kept visits/month × \$150 = \$3,750/month — more than your salary. Quote this maths in proposals.

KPIs you own weekly: confirmation rate >90%, no-show <8%, same-day fill >70% of cancellations, referral closure <14 days >85%, inbox response <4 hrs 100%, chart prep 100% by 4pm day before, recall completion trending up. Dashboard these in one Sheet — managers promote metric owners.

KPIs you own weekly: confirmation rate >90%, no-show <8%, same-day fill >70% of cancellations, referral closure <14 days >85%, inbox response <4 hrs 100%, chart prep 100% by 4pm day before, recall completion trending up. Dashboard these in one Sheet — managers promote metric owners.

3. Patient Communication Essentials: Warm, Verified, Written

Every call, SMS, and portal reply follows **Greet-Verify-Listen-Summarize-Confirm in writing**.

Opener: “Thank you for contacting Oak Clinic, this is Mercy — how may I help you today?” Never “Hello?” or “Niaje?” Verify before discussing anything: “May I confirm your full name, date of birth, and best callback number?” For returning, add “And your address on file to ensure I have the right chart?” This prevents wrong-chart breaches that end contracts.

Listen without interrupting 30 seconds, note keywords (pain scale, onset, fever, insurance change). Empathy + control: “I understand you’re worried about the wait with your pain — let me check the earliest options for you right now, including telehealth today.” Avoid “Calm down” or “That’s policy.” Offer two options, not dead ends: “I have Thu 2pm in-person or Wed 10am video — which suits you better?”

Close with read-back + written summary: “To confirm: Wed 10am Eastern / 5pm Nairobi with Dr. Lee via video — link + prep SMS and email sent. Reply YES to confirm. Anything else I can do today?” Send SMS + email within 5 minutes. Log in EHR: date/time/channel/outcome/initials. No log = not done in audits.

Voicemail rule: name + clinic + callback only, never diagnosis or results. “Hi Mrs. Smith, this is Mercy from Oak Clinic — please call us back at +1-... about your appointment tomorrow. Thank you.” Texts: approved system only (Weave, SimpleTexting), never personal WhatsApp for PHI.

Portal: reply <4 hrs, urgent flagged to phone immediately.



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Kenyan advantage: you already counselled anxious mothers in Kiswahili with patience. Transfer that warmth, slow to 90% speed for US elderly, spell with NATO ("M as in Mary"), and always ask "How do you pronounce your name correctly?" Respect builds reviews.

4. Appointment Scheduling Mastery: The Revenue Engine

4.1 Visit types, durations, and buffers

Standard template (confirm with each employer, then enforce): New patient 40 min, established follow-up 20, urgent/same-day 15 with buffer, annual physical 60, mental health intake 50 + follow-up 25, procedure 60, telehealth add 5 min tech buffer. Leave 10 min provider admin per hour (12–1pm blocked for inbox). Never stack new patients back-to-back without break — overruns cascade.

Ask reason for visit in 10 seconds to slot correctly: "Briefly, what's the main reason — follow-up for blood pressure, new cough/fever, or referral review?" Chest pain, breathing difficulty, bleeding, suicidal thoughts → urgent protocol + escalation, not routine 3-week slot. Rash with fever in child → same-day. Med refill no symptoms → 20-min or portal per SOP.

4.2 The 7 details every booking must capture

Reason + urgency, full name + DOB, phone + backup + consent to text, email for link/forms, insurance (member/group/payer) + referral status, language/interpreter/video vs in-person + address/link, and time-zone confirmation. Missing one causes failure: no backup = lost when line drops; no referral = denied visit + angry bill.

Script new booking: "Mr. Otieno, I have Tue 14 May 10:30 AM Eastern / 5:30 PM Nairobi, 40-min new-patient video with Dr. Lee. I've noted cough 2 weeks + insurance Aetna ending 8841 + backup +254... Is that correct? Link + intake SMS/email coming now — please upload ID/insurance + complete forms by Mon. Reply YES. Anything else?"

4.3 Rescheduling, cancellations, and no-show recovery that fills calendars

Reschedule offer-two: "Dr. Patel must move Thu 2pm ET — I can do Fri same time or Wed 10am — which is better? ... Confirmed Fri 2pm ET / 9pm EAT, SMS sent, waitlist updated. Sorry for the change — anything else?" Fill every cancellation within 30 min from waitlist priority: urgent first, then longest-waiting, then FIFO. Call, not just SMS: "Hi Ms. Wanjiku, a 10:30am tomorrow opened — you're first on waitlist, shall I hold 2 hours?" Document consent.



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No-show protocol: call within 15 min ("We missed you at 10:30 — are you okay? Can I rebook within 7 days?"), SMS + email, log reason (transport, forgot, tech, cost, fear), offer barrier fix (telehealth, reminder opt-in, transport info). Weekly review reasons — if "forgot" >40%, strengthen 48h+24h reminders; if "transport," push video option. Target rebook >60% of no-shows within 7 days.

US date trap: 05/06/2026 is May 6 US, 5 June UK. Always write "14 May 2026." AM/PM trap: confirm "10:30 in the morning Eastern" for elderly. Zone trap: every invite title "10:30 AM ET / 5:30 PM EAT — Dr. Lee Video."

5. Calendar Management for One and Two Doctors

One source of truth: Google or Outlook shared calendar, colour-coded green confirmed, yellow tentative/pending insurance, red cancelled/no-show, grey blocked/admin. No side notebooks. Descriptions carry handover: "Smith, J DOB 1980-05-14 | New 40 video | +1-... | Aetna 8841 verified | Intake received | Link ... | Prep: labs attached." Anyone covering you can run day from calendar alone.

Daily prep (4pm day before): audit tomorrow — forms missing? insurance unverified? consent unsigned? link tested? gaps flagged? Send doctor 7am huddle email: "Dr. Lee — Today 14 (2 new, 12 follow-up, 3 video), 1 gap 11:20am offered to waitlist (2 waiting), 3 links tested, 1 referral needs signature, 1 high-risk (chest-pain follow-up) flagged. Prep complete." Protect 12–1pm admin, no bookings. Friday: pre-fill next Mon–Tue gaps from waitlist, confirm weekend coverage.

Two-doctor juggling: separate colours per provider, never drag across without verifying duration + location (Dr. A in-person Room 2 vs Dr. B video). Buffer travel/telehealth switch 10 min. If both need urgent add-on same slot, escalate with options: "Dr. A urgent can take 11:20 gap, Dr. B urgent can take 2pm cancellation — approve?" Log decisions. End-of-week audit: utilization %, gaps filled %, double-book near-misses, fixes.

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6. Patient Follow-Ups That Retain Patients and Close Care Gaps

Follow-up is where average VAs disappear and great ones earn raises. Doctors remember who chased labs that caught kidney injury early. Patients return to practices that check “How are you feeling after yesterday?”

6.1 Three cadences to master

24-hour post-visit: SMS + portal within 24h: “Hi Mrs. Akinyi, checking how you feel after yesterday’s visit with Dr. Patel. Reply if you need refill help, referral update, or have worsening symptoms — for chest pain/breathing difficulty call 911 / go to ER immediately and inform us. Need anything else?” Log reply or no-reply. Flag worsening to clinician same hour.

Lab and referral chase Day 3-7-14: Day 3 nudge if lab not done (“Lab slip for HbA1c expires soon — nearest LabCorp at ... open 8am–5pm, no appointment needed. Need transport help?”), Day 7 call + SMS, Day 14 escalation to clinician + documented barrier (cost, fear, transport) + alternative plan. Same for referrals: sent → receipt confirmed? → patient booked? → attended? → consult note retrieved? → PCP reviewed? Close loop <14 days. Tracker columns: Patient | From→To | Date sent | Auth? | Appt date | Attended? | Note received? | Status | Owner | Next follow-up date. Review every Monday huddle — open >14 days red.

Chronic and preventive recall 30-60-90 days: Diabetes/HbA1c every 3–6 months, hypertension 3 months, mammogram/Pap annual, childhood vaccines per schedule. Pull recall list monthly, SMS batch + calls for high-risk, offer telehealth + evening slots to overcome transport/work barriers. Document outreach attempts — US quality measures (HEDIS) and UK QOF pay practices for recall completion, so your work drives revenue.

6.2 Logging that proves work

Every attempt: date/time/channel/person/outcome/next date. “12 May 4:10pm EAT called +1-... (no answer, voicemail left), SMS sent with lab address, retry 14 May 9am ET — MK.” No log = not done. Weekly report: follow-ups attempted, reached, completed, barriers top 3, actions. If “cost” blocks labs, note for social worker; if “fear,” arrange nurse call.

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7. Referral Coordination: The Skill That Makes Doctors Keep You Forever

US referrals leak 30–40% without closer — patient never books, note never returns, PCP unaware. Closer VA = revenue + safety. Workflow 8 steps, no skipping:

1. Receive order with urgency (routine <14 days, urgent <72h, STAT same-day) + reason + specialist type + clinicals (notes, labs, imaging).
2. Verify insurance: in-network specialist? auth needed? deductible met? If out-of-network, propose 2 in-network alternatives to doctor before booking — saves denial.
3. Send referral via EHR/fax/portal with cover sheet + callback verification + clinicals attached. Confirm receipt within 24h (call if no ack).
4. Book patient: offer-two times, dual-zone, telehealth vs in-person, transport needs, interpreter. SMS + email packet with address/link, prep, what to bring.
5. Remind 48h + 24h, confirm attendance.
6. Day after appt: retrieve consult note (portal/call). If no-show, rebook in 7 days + inform PCP.
7. Route note to PCP pool with summary: "Cardiology saw Smith 12 May — echo ordered, f/u 4 weeks — please review."
8. Close loop in tracker + EHR, notify patient of next step.

Packet checklist: demographics + DOB + contacts, reason + urgency, PMH/meds/allergies, relevant labs/imaging, insurance + auth number, PCP contact, patient language. Rejections are 90% missing clinicals — attach before sending, call to confirm legibility.

Example tracker row: Smith, J 1980-05-14 | Patel→Cardiology Lee | Sent 10 May | Auth AET-8841 exp 10 Aug | Appt 20 May 2pm ET | Receipt confirmed 11 May | Note pending | Status: reminded 18 May | Owner: MK | Next: retrieve 21 May. Review reds every Mon. Report closure % monthly — 55%→88% is promotion story.

8. Inbox, Email, Telephone, and Onboarding Systems

Triage 3 buckets twice daily: **Urgent now** (red-flag symptoms, suicidal thoughts, chest pain, baby fever, severe bleeding — phone + EHR flag + clinician alert in 15 min, advise ER), **Today** (scheduling, refills without red flags, forms — reply <4 hrs), **This week** (records, non-urgent admin — reply <24h with deadline). Aim inbox zero meaning every message decided, not deleted. Use canned responses personalized: "Hi {Name}, confirming {date time zones}..." but edit name/time/link every time — wrong-name macro is worse than slow.





Email template: Subject Confirmed: Appt Tue 14 May 10:30 AM ET / 5:30 PM EAT – Dr. Lee (Video) + Dear Mr./Ms. + 3 short paras (what/when/where, prep, action Reply YES) + signature with hours both zones. Escalation to doctor: Subject [Urgent – Review Today] Portal chest pain 2:10pm ET – Smith, 58M + SBAR + what you told patient + what you need.

Phone: headset, EHR open, smile audible, verify 2 identifiers, summarize, commit time, send written confirmation. Onboarding packet for new: welcome letter + intake + ID/insurance upload link + portal invite + telehealth consent + tech guide + FAQ + “what to expect” video. Checklist 8 items, completeness >95% before first visit or visit fails. Offer 10-min tech check for elderly — cuts no-shows 30%.

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9. Escalation: Act in 15 Minutes, Never Diagnose

Red-flag list to memorize: chest pain/pressure, difficulty breathing, facial droop/slurred speech, severe bleeding, suicidal/homicidal thoughts, severe allergic reaction (lip swelling, wheeze), baby <3 months fever >100.4°F/38°C, severe dehydration, labour pains with bleeding. Any of these via phone/portal/SMS → urgent path.

Script urgent: “Thank you for telling us — this sounds urgent. Please call 911 now (US) / 999 (UK) / 000 (Australia) or go to ER immediately. I am alerting our clinical team right now and will stay on the line while you arrange help if you wish. May I confirm your location and callback?” Then: phone clinician pool + EHR urgent flag + task with timestamp + document what you advised + follow up in 30 min. Never say “It’s probably nothing” or “Take Panadol and wait.”

Non-urgent clinical (“Can I double my BP dose? Is this rash HIV?”): “I understand you want an answer quickly — I’m not qualified to advise on medication/diagnosis as your coordinator, but I’ve marked your message urgent for Dr. X with your details, expected reply by 4pm ET today. For worsening symptoms — fever, breathing difficulty, spreading rash — please seek urgent care now. Can I help with booking or records while you wait?” Log escalation time + recipient. Your job is speed + safety, not answers.





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Test yourself weekly with 5 mock messages — classify urgent/today/week and write exact reply + escalation. Speed matters: audit shows >30-min delay for chest pain triples risk. Set phone alarm for “recheck urgent pool every 30 min in shift.”

10. Practical Simulations: Prove You Can Do the Job (Portfolio Required)

Do all 7, log with screenshots/recordings, no real PHI — use mock names. Submit folder AfyaDesk/Module3/ with PDFs + 2-min audios + reflection.

1. **Book new patient:** 40-min video, insurance Aetna mock, intake packet sent, eligibility noted, dual-zone invite. Deliverable: calendar invite screenshot + email/SMS copy + EHR admin note.
2. **Reschedule + fill gap:** Move Thu 2pm to Fri, fill Thu gap from 3-person waitlist priority (urgent first). Deliverable: before/after calendar + 2 consent SMS + tracker update.
3. **Reminders for 10:** SMS + email batch 48h + 24h, 7 confirmed, 2 rescheduled, 1 no-answer called twice. Deliverable: reminder log + confirmation % calc.
4. **Inquiry call:** Price + availability + insurance question, 3-min recording. Scored on greeting, verification, two options, read-back, written follow-up. Submit audio + self-score.
5. **Chest-pain escalation:** Portal message “tight chest 20 min” — write reply + SBAR to doctor + log times. Must be <15 min simulated, ER advice included, no diagnosis. Deliverable: reply copy + task screenshot + reflection on what felt hard.
6. **No-show follow-up x3:** Different reasons (forgot, transport, cost) — call script + rebook + barrier fix + log. Target 2/3 rebooked in 7 days.
7. **Two-doctor day:** 22 appointments across Dr. A (in-person) + Dr. B (video) with 2 urgent add-ons, no conflict, buffers kept, huddle email sent. Deliverable: calendar day view + huddle email + audit note (utilization %, gaps filled).

Reflection 300 words: what almost broke (zone? waitlist order? escalation fear?), what system fixed it, what SOP you'd improve with data. Distinction: adds referral closure % improvement idea with maths.

Common Mistakes That Fail Trials

Booking without reason/duration leading to 40-min new in 20-min slot and 60-min delay; forgetting zone causing 7-hour miss; giving reassurance “That’s normal” instead of escalation; sending referral without clinicals causing rejection loop; discussing PHI on personal phone; leaving



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cancellations unfilled while waitlist waits; no EOD so manager assumes no work. Pin this list above desk for 90 days.

Self-Check Quiz (12 Questions)

1. List 4 DOs and 4 DO NOTs of MVA scope with interview-safe wording.
2. Map 9-step workflow and name your KPI at steps 3, 4, 7.
3. What 7 details must every booking capture? Write booking script for new video.
4. How do you fill cancellation in 30 min from waitlist priority?
5. Write no-show call + SMS for “forgot” vs “transport” barrier.
6. Draft daily huddle email for 14-patient day with 1 gap + 1 pending signature.
7. Explain Day 3-7-14 chase with example tracker row.
8. List 8-step referral loop and packet checklist.
9. Triage 5 mock inbox messages into urgent/today/week with replies.
10. Write escalation reply + SBAR for suicidal ideation portal message.
11. Draft onboarding checklist 8 items + tech-check offer script.
12. Which 2 simulations were hardest and what system will you keep?

Pass 9/12. Redo simulations for any miss — reading without doing fails interviews where you role-play live.

Key Takeaways

Great MVAs own access, accuracy, and kindness: tight calendars with zones and buffers, relentless follow-up with logs, inbox zero with safe triage, referrals closed not sent, escalations in minutes not hours. You understand medicine to schedule correctly and document precisely, but you never diagnose. Your metrics — confirmation >90%, no-show <8%, closure <14 days, response <4 hrs — are revenue and safety. Master these and every later module (terminology, privacy, billing, tech) plugs into daily work you already prove.

What's Next

Module 4 — Medical Terminology & Documentation: decode 200+ terms, avoid killer abbreviations, write SOAP-aligned admin notes, referral letters, and discharge packets without





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crossing into clinical decisions. Bring your MVA cheat-sheet — you will add terminology columns to it.

Portfolio reminder: export your 7 simulation logs as PDFs with dates and hours. Hiring managers request 5–7 work samples before offering trials, and this module gives you exactly those samples for AfyaDesk Talent Profile success and long-term client trust always reliably every single week consistently without any exception.



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